

Advise the pregnant patient

- Complete Maternity Case Record and give to patient, remind patient to bring it to every visit and when in labour.
- Encourage patient to register on MomConnect (dial *134*550#) to receive messages to support her and her baby during pregnancy, childbirth and baby's first year.
- Advise patient to only take medications prescribed by nurse/doctor who knows she is pregnant. If she is unsure, advise her to check with nurse/doctor.
- Alert patient to the risks of smoking and drinking alcohol and urge to stop. Support patient to change ↗ 177 and refer patient to available helpline ↗ 178.
- Discuss safe sex. Advise patient to use condoms throughout pregnancy and have only 1 partnership at a time.
- Discuss contraception choice for after delivery ↗ 154.
- Educate about signs of early labour and pregnancy emergency: persistent headache, blurred vision, abdominal pain (not discomfort), drainage of liquor, vaginal bleeding, reduced fetal movements.
- From 30 weeks, ensure patient knows where she is going to give birth and check if transport arrangements have been made should she go into labour.
- Discuss infant feeding:
 - Encourage exclusive breastfeeding for 6 months, regardless of HIV status: this means that baby gets only breast milk (no formula, water, cereal) and if HIV-exposed, infant prophylaxis.
 - From 6 months, introduce food while continuing with feeding choice. Continue breastfeeding until 2 years for all, ensuring that HIV positive mother is adherent on ART and virally suppressed.
 - If mother chooses to exclusively formula feed, check that mother will be able to afford it long-term, has access to clean boiled water, and that it will be acceptable (i.e. no disclosure issues).

Health for All

↗ 55

Treat the pregnant patient

- Give **folic acid** 5mg daily up to 13 weeks gestation. If on anticonvulsants, family history or previous baby with neural tube defect, continue folic acid throughout pregnancy.
- Give iron¹ according to Hb:
 - If Hb ≥ 10, give **ferrous sulphate compound BPC** 170mg daily or **ferrous fumarate** 200mg daily throughout pregnancy. If daily iron not tolerated², give instead **ferrous sulphate compound BPC** 340mg once weekly with food or **ferrous fumarate** 400mg once weekly with food throughout pregnancy.
 - If Hb < 10, give **ferrous sulphate compound BPC** 170mg 12 hourly with food or **ferrous fumarate** 200mg 12 hourly with food.
 - Continue for 3 months once Hb ≥ 10, then once daily throughout pregnancy.
- Give **elemental calcium** 1g daily (given as 3 tablets of calcium carbonate (420mg tablets), 12 hourly) to reduce the risk of pre-eclampsia.
 - If previous pre-eclampsia, discuss giving low-dose aspirin from 6 weeks' gestation (preferably before 16 weeks) with specialist.
- If first pregnancy, give **tetanus toxoid** (TT) 0.5mL IM into arm (when available, give instead **Tdap**). If < 5 previous tetanus vaccinations⁴ in lifetime documented, catch up vaccinations.
- Give **influenza vaccine** 0.5mL IM if at time of annual campaign.
- Check that patient is up to date with COVID-19 vaccination.
- If **gestational hypertension**:
 - Start **methyldopa** 250mg 8 hourly and titrate up to 750mg 8 hourly if needed.
 - Review weekly, check for new symptoms, BP, urine, weight, SFH and fetal heart/movements ↗ 161.
 - Refer at 38 weeks for delivery at hospital.
- If **HIV positive**: start or continue ART and check if prophylaxis (e.g. co-trimoxazole preventive therapy or TB preventive treatment) needed ↗ 113.
- If in **malaria** area, discuss need and choice of malaria prophylaxis with specialist.

Review the pregnant patient at 20, 26, 30, 34, 36, 38, 40 weeks. If undelivered, also review at 41 weeks.

Treat the HIV positive patient in labour

- If on ART, continue ART throughout delivery. Check viral load, regardless of when last done, and review results at 3-6 day postnatal visit.
- If not on ART, give together single dose **NVP** 200mg as early as possible in labour and single dose (TLD) **TDF/3TC/DTG** 300/300/50mg.
- Give ideally during early labour, and urgently if delivery is imminent.
- Start mother on ART next day ↗ 114. Give mother 2 months ART supply.
- Decide HIV transmission risk of HIV-exposed baby and give infant prophylaxis according to risk ↗ 168.

DTG - dolutegravir; **FTC** - emtricitabine; **NVP** - nevirapine; **TDF** - tenofovir; **3TC** - lamivudine, **TLD** - TDF/3TC/DTG or tenofovir/lamivudine/dolutegravir

Note: fill 'C#DELIVERY' in the code field on blood request form so VL not rejected.

Give routine postnatal care to mother and baby → 164.

¹If possible, avoid taking iron within 4 hours of taking calcium or methyldopa and within 2 hours of milk and tea. If on dolutegravir and taking at same time as iron, take with food. ²Abdominal pain, nausea, vomiting, constipation. ³If on dolutegravir and taking at same time as calcium, take with food. ⁴Tetanus vaccinations include DTP, DTP-Hib, DTaP-IPV/Hib, TD or TT.